

VDACL Documentation Guidelines



Writing at Work

As a support worker at VDACL, there are many situations where you will be required to complete documentation:

- Daily charting notes, goal progress notes, weight charts, health notes, medication records, communication book entries
- Incident reports, behaviour charts
- Emails, letters, texts, phone messages
- Time off requests, mileage claims, checklists

Why so much documentation?

We do three types of writing at work:

- 1) Documentation for individuals in care
- 2) Information for colleagues
- 3) Records and administrative requirements

It all has one basic purpose: to improve the quality of care and support we provide.



The Support Team

Support work is a partnership between people working in different places at different times.

Everyone involved needs up-to-date, accurate and quite detailed information about:

- The person
- Their care plan
- What other team members are doing



Writing is the best way to share information between the support team.

What we write keeps them in touch with the person and their needs. The team members rely on our records and reports to know when the person's needs are changing.

Who Reads our Writing?

What we write at work may be read by many different people:

- The person, their family
 - People who use care services have a legal right to see our information about them.
- Colleagues and care givers
 - Other support workers, program coordinators read what we write to understand the person's care needs.
- Other partners (e.g. social workers, doctors)
 - The support team base care decisions on what we write.
- Inspectors, Lawyers, police, and other officials
 - What we write may be (and often is) used in complaints, investigations, and legal cases.

In any one of these situations, you may be asked to explain what you have written. That is why it is important to write clearly, factually, accurately, and respectfully, and considering the person wherever possible.

Did you know? A record is any recorded information.

We create a record whenever we write down information, e.g. care plan notes, chart entries, phone messages, emails etc.

Documentation Essentials

The following are essential to ensure successful documentation. Writing must be:

- **Timely:** done as soon as possible (while fresh in our memory)
- **Easy to read:** clear handwriting, accurate spelling and grammar
 - Use **blue** or **black** ink in all documentation and **red** ink for medical - never pencil
 - Avoid spare lines and gaps between handwritten entries
- **Dated, timed, and signed :**
 - Use 24-hour clock (16:00 instead of 4:00pm)
 - Sign entries with printed name and/ or signature/ initials
- **Complete :** all the necessary information
- **Accurate :** correct details (Names, times, dates, etc.)
- **Clear:** plain English that everyone (including the person) can understand, no abbreviations, no jargon
- **Factual:** just what happened, no opinions, comments, assumptions, or speculation
- **Respectful :** of the people in our care: no judgements, no personal comments, reflects dignity and respect

*Speak to the Program Coordinator or Manager if you have any questions regarding these guidelines.

Confidentiality

Our records often contain personal information about the people we care for.

The law says that we must treat personal information as confidential.

That is why VDACL has a policy on confidentiality, including how to document and store confidential information.

In addition, upon hire and every three years, all staff must complete CLBC's Privacy and Information Management Training.



Report Writing

When we describe the care we give to a person, we are writing a report. The Support Team uses our reports to help plan the person's care. That's why our reports must be factual.

***Read this interaction:**

You look upset.

It's Mrs. Smith, she threw a hairbrush at me.

What! Are you OK?

Yes, it missed. But she was so angry and she called me names. She called me a thieving rat!

But she likes you. What's got into her?

No idea! I just went in as normal to help her get dressed and she started screaming at me.

What did you do?

I just left. I feel really upset.

Now compare two written reports of what happened:

Report 1

Mrs. Smith behaved unacceptably today.

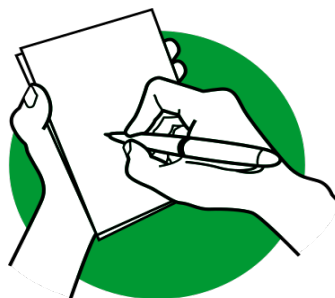
Report 2

I knocked at the door. Mrs. Smith said, "Come in", but when she saw me her face went red. She shouted, "Get out, you thieving rat!" She threw the hairbrush she was holding at me. It hit the wall beside me. I left the room at once.

Learning question - Which report would the support team find more useful - and why?

Report 1 - *Gives us no information about what happened. It is not a factual report.*

Report 2 - *Gives factual information. It says what happened, in the order it happened.*



Writing About Behaviour

We need to be mindful to write factually when we are writing about people's behaviour.

What is behaviour?

Behaviour is what a person does and says. When we are with another person, we naturally think about four things:

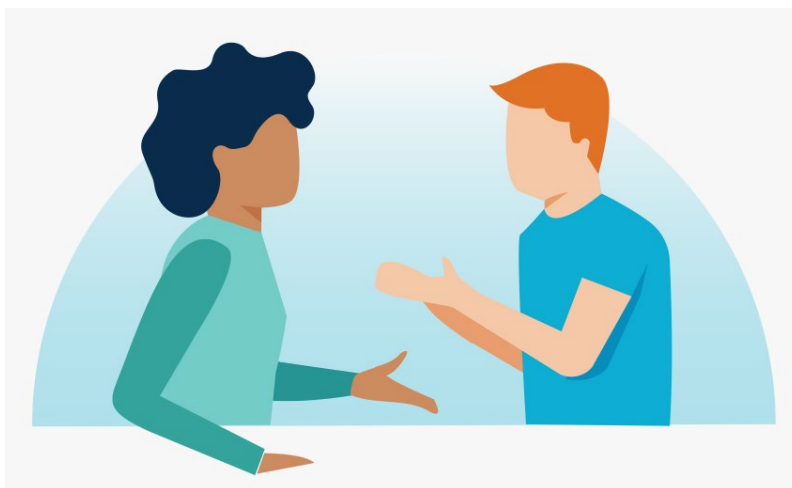
1. What is this person doing/saying?
2. Why are they doing/saying that?
3. How does it affect me?
4. What do I feel about that

Question 1 is about behaviour. This is what we should report - what we see and hear the person doing and saying. It definitely happened. It is factual information.

Question 2 is about what the person's behaviour means. Unless we actually ask the person, we cannot be sure about this. If we do ask the person, we can report what they say and make it clear that we are reporting what they told us - but that is all. If you have not asked the person, don't try to explain their behaviour in your report.

Questions 3 and 4 are about us, not the person. How relevant do you think they are?

We write reports to help the support team plan the person's care. **Our feelings are important, but they do not belong in a factual report about the person.** Feelings can lead to bias, like writing that the person was *difficult* or *challenging*. This is unhelpful to the support team and may be offensive to the person.



Is it a Fact?

What is the difference between fact, assumption, and speculation?

Fact

A fact is something that has happened, e.g. *Mr. Clark had toast for breakfast*. Facts exist in the real world. Other people can check them. Facts are objective.

If you're not sure, ask yourself these questions:

Did I see or hear it happen?

- *Yes > Then it's a fact.*
- *No > Then how do you know about it?*

Someone told me. *Did I check what they said was right?*

- *Yes > Then it is a fact.*
- *No > Then it's just hearsay. All I can write is that someone told me it happened.*

Assumption

If we accept something is true without proof, we are making an assumption (i.e. assuming)

I'm sure Mr. Clark had toast for breakfast. He always does.

In fact, today he didn't. He had fruit. *Mr. Clark had toast* is an assumption .

Speculation

If we guess the reason for something, we are speculating, e.g. *Mr. Clark was talking to himself. I think he was hallucinating.*

In fact, he was singing to himself. *I think he was hallucinating is* speculation.

Watch out! It is easy to mistake assumptions and speculation for fact, so always ensure you are recording only facts in your documentation.



Respectful Language

Always ensure that you are writing and speaking about individuals you are supporting using language that is:

- Specific
- Objective
- Respectful

Specific

A brief description that includes precise details: who, what, where and why something happened.



Objective

A brief description that is based solely on observable or verifiable facts and devoid of personal opinions.

Respectful

Language that avoids labelling or infantilizing the person.

Do Words matter?

Words can...

- Provide a clear understanding of how to approach a person
- Offer a clear description of responsive behaviour triggers, behaviour, and resolution
- Support the creation of clear plans
- Assist in evaluation of core interventions



What about responsive behaviours?

Responsive behaviours

(words, gestures, actions) have meaning and are an important means of communicating needs and concerns.

When describing responsive behaviours, it is critical that the language used be specific, objective and respectful in order to facilitate understanding and appropriate care planning.



Word Swap

Instead of... Try this...

Aggressive *"While attempting to assist to a standing position, the individual screamed and pulled staff's hair"

Agitated *"Individual leaning forward on edge of chair, voice getting louder"
*"Individual clenching jaw"

Confused *"Individual not orientated to time or place"
*"Individual repeating sounds loudly while peri-care provided"

Disruptive *"Individual repeatedly calling for help while at the kitchen table "

Word Swap

Instead of... Try this...

Exit-seeking *"Individual trying to leave the program - needs to go home to walk the dog"
*"Individual trying to leave program and states she has an appointment to get to"

Non-Compliant *"Individual declines to participate in group activity"

Resists care *"Individual declines medications by turning head away"

Restless *"Individual walking around room, does not sit down"
*"Individual does not appear calm when sitting in chair"

Word Swap

Instead of... Try this...

Screamer/ Screaming *"Individual repeatedly calling out spouse's name during morning care"

Sexually inappropriate *"Individual took off their pants and walked out of the room"

Verbally abusive *"Individual cursing when staff attempted to give medication"

Wanderer *"Individual frequently enters another individual's room"
*"Individual frequently walks about in hallways during meal times"

In addition, we want to ensure we are writing and speaking about the adults we support with dignity and respect. Consider the following:

Instead of	Consider
Diaper	Adult brief or Incontinence product
Bib	Clothing/shirt protector
Sippy cup	Glass, cup or mug

Use the **proper names** of items that support individuals' activities of daily living; avoid infantilizing them.

Avoid Comments

Earlier, we read two reports. Report 1 said: *Mrs. Smith behaved unacceptably today.*

This is not factual, detailed, or helpful information. It is the writer's comment or opinion.

What is an opinion?

Opinion is what we feel or think about something.

The staff may honestly feel that Mrs. Smith's behaviour is *unacceptable*.

That does not make it factual information. *Why not?*

Opinions are subjective. They exist only in our mind. That is why two people can hold different opinions about the same thing, e.g. *Mrs. Smith behaved unacceptably today* or *Mrs. Smith was confused and very agitated today*.

Look how comments and opinion can change the meaning and/ or interpretation of our documentation.

How we feel about something:

Mrs. Smith called me a thieving rat. Fact

I feel abused by Mrs. Smith. Feeling

In my report I write:

Mrs. Smith was abusive. Opinion/ comment

What we think about something:

Mr. White had two bowls of pasta today. Fact

I think that is too much food. Judgement

In my report I write:

Mr. White ate too much food today. Opinion/ comment

Opinion and comments do not belong in factual reports. Avoid them simply by sticking to the facts.

Specific and Complete

Always write exactly what happened, in detail and using descriptive language.

Don't write: *Mrs. Peters drank a lot of water.*

Do Write: *Mrs. Peters drank three glasses of water.*

Don't write: *We went for a short walk.*

Do write: *We walked to Main Street and back.*

Don't write: *Jamila had a good day.*

Do write: *Jamila played cards with Sharon.*

Avoid abbreviations

Abbreviations cause confusion.

- Not everyone understands them
- They may mean different things to different people

Don't write: *p/u* or *b/fast* or *NOD* **Do write:** *passed urine* and *breakfast* and *nurse on duty*.

How to correct a mistake

It is easy to make a mistake when writing notes or filling a document at work.

If this happens just **draw one line neatly through the mistake** , so it can still be read.

Write in the correction, then **add the time date and your initials** in small letters by the alteration and **write *error*** to indicate the correction was intentionally done and by whom.

error M.E. 18/05/24

Individual was visited by her ~~daughter~~ son.

This shows it was a genuine mistake, not an attempt to alter records.

- Do not use white out, black out marker, or scribbles on an error.
- Never correct someone else's error.

Forms, Charts, Checklists

Forms, charts, and checklists often require little writing, just a word or a number, the time and date, and your signature or initials.

However little writing they require, the information they collect is important for:

- Monitoring changes in health and well-being (for support/ care planning)
- Regulations and legal requirements
- Quality inspections
- How your work is organized

With your forms and charts, do you know...

- Exactly what information is required?
- Where to write the information?
- How to write it (e.g. in black ink)?
- Any special requirements (e.g. use the 24hr clock when writing times)?
- When to complete the document (e.g. at the time, end of the shift, weekly, monthly)?
- What to do with the document (e.g. leave it where it is or hand it in to the office)?
- What it looks like when it is properly filled in? (ask your supervisor to show you)
- What to do if you can't fill it in?

Learning question - How easy is it for other people to read your handwriting?

Never sign or initial something that you didn't do!



Incident Reports

We write Incident Reports to:

- Inform others
- Raise safety issues
- Create a record of events
- Learn from any mistakes
- Improve the quality of our service
- Comply with regulations
- Confirm facts during investigations



What to write:

- Name of those directly involved
- Date, time, place of the accident/ incident
- Name those who saw what happened (witnesses)
- Short, accurate, factual description stating what happened in the order it happened
- Describe what happened prior to the incident, what happened during the incident and following the incident.
- Any hazards present (e.g. *the floor was wet*)
- Any action taken afterwards (e.g. *Nurse examined Mrs. Smith*)
- Only include factual information. If including a verbal report from another person, you must make that clear.
- Remember you are writing for an audience. Many people may read this report so be factual, use correct spelling/ grammar/ sentence structure and remain professional in language and tone.

What not to write:

- Don't explain why it happened (don't write *She fell because the floor was wet.*)
- Never blame anyone (Do not say *It was the cleaner's fault for leaving the floor wet.*)
- Do not record information based on opinions, emotions, or bias.

Useful terms

Incident = anything that unusual that happens to the people in care, visitors, or workers - including a near miss, odd behaviour, and conflict.

Accident = event causing harm, loss, or damage to the people in care, visitors, or workers.

Near miss = event that could have caused harm, loss, or damage, but luckily did not.



Remember to Report and Document Right Away!

People have limited short-term memory. Things that may diminish or corrupt memories include:

- Information about incidents being discussed with others before it is documented.
- How much time has elapsed before recording what you observed - the longer the time between, the more likely intervening experiences have “contaminated” the accuracy of your memory.
- Your bias, prejudice, and pre-conceived notions about people.

Emails

Emails have a set layout :

Write the topic of your message in the subject line. This tells the person receiving the email what your message is about.

- Include any action you want the person to take: *Care plan review- please attend.*
- Greet the recipient by name: *Dear Chris,*
- Keep the message as short as possible but make sure to include all the information the recipient will need to understand and respond to your message.
- Highlight key information with bullet points.
- Use proper sentences. Avoid abbreviations, initials and jargon (they often confuse people).
- Close by saying what action you want the recipient to take: *Please confirm you can attend this meeting.*

Email etiquette

Writing in capitals is seen as shouting in email.

Don't write PLEASE RESPOND ASAP **Do write** *Please respond as soon as possible.*

Learning questions - Is an email a record? (answer on Page 2!)